

Sponsoring Organisation:		Implementation Date:	01 December 2015
DH Informatics Directorate		Subject: Address Input and Display for Clinical Systems within the NHS in England	
DATA SET CHANGE NOTICE			
This DSCN informs users of the approval of a new information standard by the Information Standards Board for Health and Social Care (ISB). It was presented at ISB’s meeting on 24 September 2009 and subsequently approved having met the required conditions.			
Summary: This user interface standard specifies the rules for the display of addresses, input of address data elements, input of UK addresses, the UK address finder and input of non-UK addresses. Organisations developing or upgrading applications for which the standard is in scope MAY implement the address input and display standard immediately and SHOULD do so at their next major system release. All clinical systems within the NHS in England MUST conform to the address input and display standard by 01 December 2015. Patient safety risks associated with the address input and display standard are specified in the safety case accompanying the standard. Organisations that develop, supply, deploy and use clinical systems must identify and manage any risks in current systems and mitigate implementation risks for which responsibility for mitigation has been transferred to the supplying or deploying organisation. The process for managing safety risks is detailed in NHS information standards for the application of patient safety risk management to the manufacture, deployment and use of health software. The standard can also be used to inform design of user interfaces in other systems and devices provided any safety risks are identified and managed, for example, risks related to reduced screen size.			
Other Datasets / Return Affected: None			
Related DSCNs: ▪ DSCN 18/2009 - Application of Patient Safety Risk Management to the Deployment and Use of Health Software. ▪ DSCN 14/2009 - Application of Patient Safety Risk Management to the Manufacture of Health Software. ▪ AN/0904 – Demographics Display and Input for Clinical Systems in the NHS in England. ▪ DSCN 09/2010 – Patient Banner for Clinical Systems within the NHS in England.			
Impact of Change: Service: Major			

DATA SET CHANGE NOTICE

Reference No:	DSCN 01/2010
Version No:	1.0
Subject:	Address Input and Display for Clinical Systems within the NHS in England
Type of Change:	Introduction of a new approved Information Standard
Implementation Date:	01 December 2015
Business Justification:	To make display, input and user interaction consistent across all clinical systems within the NHS in England in order to increase patient safety by maximising clinical utility and minimising reading and transcribing errors.

Introduction

Displaying and inputting unambiguous information in a consistent format is a core element in ensuring effective patient care. It is vital that healthcare IT users correctly interpret and input information relating to all aspects of patient care and management. This user interface standard specifies the rules for the display of addresses, input of address data elements, input of UK addresses, the UK address finder and input of non-UK addresses.

The standard has been developed as part of the Common User Interface (CUI) programme within the Technology Office of the Department of Health Informatics Directorate. This programme is addressing the safety risks posed by variation across the NHS in the way information is input and displayed. Its aim is to support healthcare IT users in correctly inputting and interpreting information relating to all aspects of patient care and management.

Background

NHS clinical and healthcare applications store and display addresses in various contexts. For example, the address may be a patient's residential address, or the address of a doctor's surgery. Addresses play an important, though secondary, part in patient identification. Where only a patient's name and gender are known, the address can help identify the correct patient from a list of matching results. The address must therefore be easily accessed, such as from the Patient Banner, as described in the CUI Patient Banner standard.

Addresses need to be stored by clinical applications, displayed on monitors and printed on referral letters and envelopes. There is therefore a need to identify best practice regarding address input and display and to promote its correct usage across all NHS clinical and healthcare applications. This ensures that users have a consistent experience and maintains a high standard of data quality across the NHS.

Addresses must be accurate to enable delivery to, collection from or visits at the correct location. Therefore, it is essential that all NHS clinical and healthcare applications provide an easily-readable and consistent display format for address information. Promoting patient safety is of primary importance, so it is critical that addresses are presented in a clear form that minimises ambiguity and reading and transcribing errors. An address that is not displayed in a clear form can be misread, or not understood at all. This could have patient safety issues if, for example:

- A patient needs to be sent a letter about an appointment.
- Test results need to be sent to a hospital or General Practitioner (GP) surgery.
- A next of kin needs to be contacted about the patient.
- A consultant needs to be contacted about a patient.

If clinical systems made by different developers vary in the way they display addresses, there is a risk of misinterpretation by healthcare professionals moving between those systems. This has the potential of leading to Patient Safety Incidents, as defined by the National Patient Safety Agency (NPSA).

Having a standard method of displaying addresses makes the design and development of clinical systems easier and quicker. This benefits the NHS by having new systems available earlier and at lower cost.

Details of the Standard

The standard is published with associated guidance in a Common User Interface Design Guide. It is applicable to user interfaces displayed on desktop or laptop computers. It is assumed that, at a minimum, these computers are capable of operating at a minimum display resolution of 1024 x 768, and have a keyboard and pointing device.

The following items are covered in the guidance:

- Address display.
- Address input data elements.
- UK address input.
- UK address finder.
- Non-UK address input.

Aspects that are **out of scope**:

- Validation of an entered address as a real address.
- Validation that a given address is that of the stated person.
- Multi-language applications.
- Display styles.
- Reduced-size form factors.
- Data storage and transmission.
- Data history and provenance.
- Address types.
- Method of providing help text and user messages.
- Address picker.
- Form design.

Safety Case

Clinical safety requirements and objectives related to the input and display of addresses in clinical systems are documented in the safety case that accompanies this standard (www.isb.nhs.uk/documents/cui). Organisations that develop, supply, deploy and use clinical systems are required to identify and manage any risks in current systems until the standard can be implemented. They must also mitigate those live implementation risks outlined in the safety case for which responsibility for mitigation has been transferred to the supplying or deploying organisation.

The process for assessing and managing safety risks is detailed in NHS information standards for the application of patient safety risk management to the manufacture, deployment and use of health software (<http://www.connectingforhealth.nhs.uk/dscn/dscn2009/data-set-change/>), i.e.:

- Application of Patient Safety Risk Management to the Manufacture of Health Software (DSCN 14/2009); and
- Application of Patient Safety Risk Management to the Deployment and Use of Health Software (DSCN 18/2009).

Conformance

For full conformance to be demonstrated, the elements marked as 'Mandatory' in the Common User Interface Design Guide must be complied with and there must have been mitigation of those live implementation risks outlined in the safety case for which responsibility for mitigation has been transferred to the supplying or deploying organisation.

Timescales for Implementation

FRAMEWORK	Health and Social Care Personnel	Organisation	IT Suppliers
Effective Date “may use”	Immediate	Immediate	Immediate
Implementation Date “must use”	When available in system	At next major system release	At next major system release
Conformance Date “must be used effectively and assessed for use”	01 December 2015	01 December 2015	01 December 2015
Superseded Date (of prior standard) “stop using prior standard”	Not applicable	Not applicable	Not applicable

Effects on Other Information Standards:

None

Sponsor Details

Ken Lunn, Director of Data Standards and Products
Department of Health Informatics Directorate
Technology Office

Further Information and Support

- CUI Address Input and Display Design Guide - www.cui.nhs.uk
- Clinical Safety Case - www.isb.nhs.uk/documents/cui
- CUI website - www.cui.nhs.uk
- Standards documentation – www.isb.nhs.uk/documents/cui
- Queries - cui stakeholder.mailbox@nhs.net

Appendix

This DSCN must be read in conjunction with the Clinical Safety Case and Closure Report for Demographics / Information Input and Display, which can be found at the following link: www.isb.nhs.uk/documents/cui

Table Notes:

1. Relevant organisations are those organisations as defined in the standard who must take direct action to implement the standard
2. IT Suppliers are all suppliers to the organisations listed at ¹ who supply functionality pertinent to that standard
3. **Effective Date** is the date from which a new standard can be used but may not be mandatory. This might facilitate piloting, for example, or enable time for system functionality development. At this point, **you “may use” the standard.**
4. **Implementation Date** is the point from which the new standard becomes mandatory. Ideally, it inherently implies organisations use appropriate systems i.e. the date is the same for organisations and suppliers. However, there may be circumstances where interim workarounds are required i.e. the date is different for organisations and suppliers. At this date, **you “must use” the standard.** Where the standard demands data is submitted centrally, sub components of implementation date (and possibly ‘effective date’) are:
 5. **Collection Start Date** – this is the date collection of data must begin
 6. **First Submission Date** – this is the date of first submission of data centrally
 7. **Reporting Period / Submission Cycle** – If the standard calls for further collection and submission at defined intervals, this cell provides text of the reporting period (e.g. calendar month, financial year) and the submission cycle (e.g. submit data monthly on the 10th working day of the subsequent month).

8. **Conformance Date** is the date from which the service and IT system suppliers must use the standard as envisaged i.e. using appropriate IT solutions rather than interim workarounds and, if the standard requires it, an independent, authoritative body or legitimate internal audit would conduct a conformity assessment with the expectation of full conformance by all relevant parties. It is the ***“must use standard effectively and assessed for use”*** date
9. **Superseded Date** of the prior standard sets the date at which the prior standard is replaced by the new standard i.e. the prior standard must no longer be used. This date will apply only where there was a pre-existing standard made redundant by the new standard. It might be different from preceding dates in the framework if, for example, a new and old standard run in parallel for a period. It is the date from which you ***“stop using the prior standard”***.